

Colon Cancer — Clinical Trial & Treatment Timeline

Alert Window: Last 30 Days · Generated: 2026-03-09 20:41:31

Situation Brief

Recent developments in colorectal cancer treatment highlight significant advancements for specific patient populations, particularly those with BRAF mutations and CDK12 deficiencies. The FDA's approval of the Encorafenib and Cetuximab combination offers a new targeted therapy for Stage IV metastatic colorectal cancer, while ongoing trials explore innovative combinations for locally advanced and unresectable cases. These findings may influence treatment decisions, emphasizing the importance of genetic profiling in therapy selection. Moving forward, attention should be given to the outcomes of the various trials, particularly those involving novel combinations and their impact on patient care.

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Situation Summary

Events Detected: 8
High Significance: 1
Top Category: New Trial
Primary Stage: Stage IV
Primary Target: General
Research Activity: Active Monitoring

Event Timeline

EVENT CLUSTER — Adding Encorafenib and Cetuximab to FOLFIRI Shows Benefit in BRAF-Mutant Colorectal Cancer

MEDIUM

ALL STAGES

BRAF

TRIAL RESULTS

VERIFIED

Mar 9 19:40 · immediate · The ASCO Post

A recent study shows that combining Encorafenib and Cetuximab with the chemotherapy FOLFIRI improves treatment outcomes for patients with BRAF-mutant colorectal cancer.

1 source(s) reporting · cluster 1 of 8

EVENT CLUSTER — FDA Approves Encorafenib (Braftovi) Combination for Metastatic Colorectal Cancer

HIGH STAGE IV BRAF TARGETED THERAPY UNVERIFIED

Mar 9 19:40 · immediate · [Oncology News Central](#)

The FDA has approved the combination of encorafenib (Braftovi) for treating Stage IV metastatic colorectal cancer with BRAF mutations, offering a new targeted therapy option that may improve outcomes for patients with this specific genetic profile.

1 source(s) reporting · cluster 2 of 8

EVENT CLUSTER — Low-Dose Radiotherapy to Sensitize Pucotenlimab Plus CAPEOX for pMMR Locally Advanced Rectal Cancer

MEDIUM STAGE IV GENERAL NEW TRIAL CORROBORATED

Mar 9 19:40 · immediate · [clinicaltrials.gov](#)

A new clinical trial is exploring the use of low-dose radiotherapy to enhance the effectiveness of the drug pucotenlimab combined with CAPEOX chemotherapy for patients with locally advanced rectal cancer, potentially improving treatment outcomes.

1 source(s) reporting · cluster 3 of 8

EVENT CLUSTER — A Clinical Study of Iparomlimab and Tuvonralimab Combined With Bevacizumab and Alternating Triweekly CAPOX/mCAPIRI Regimen as First-line Treatment for Unresectable Advanced Colorectal Cancer

MEDIUM STAGE IV VEGF NEW TRIAL CORROBORATED

Mar 9 19:40 · immediate · [clinicaltrials.gov](#)

A new clinical trial is exploring a combination treatment of Iparomlimab, Tuvonralimab, and Bevacizumab with chemotherapy for patients with advanced colorectal cancer, potentially offering a new first-line option for those with unresectable disease.

1 source(s) reporting · cluster 4 of 8

EVENT CLUSTER — Everolimus in CDK12-Deficient Metastatic Colorectal Cancer (EVER-RECODE)

MEDIUM STAGE IV GENERAL NEW TRIAL CORROBORATED

Mar 9 19:40 · immediate · [clinicaltrials.gov](#)

The EVER-RECODE trial is investigating the use of everolimus for patients with Stage IV colorectal cancer that has CDK12 deficiencies, potentially offering a new treatment option for this specific group.

1 source(s) reporting · cluster 5 of 8

EVENT CLUSTER — Effect of Perioperative Lidocaine or High-Dose Dexamethasone on Immune Response in Colon Cancer Surgery (PILDI Study)

MEDIUM

STAGE III

GENERAL

NEW TRIAL

CORROBORATED

Mar 9 19:40 · immediate · clinicaltrials.gov

The PILDI Study is investigating whether perioperative lidocaine or high-dose dexamethasone can enhance the immune response in patients undergoing surgery for Stage III colon cancer, potentially improving recovery and treatment outcomes.

1 source(s) reporting · cluster 6 of 8

EVENT CLUSTER — Neoadjuvant Chemoradiotherapy Plus Tislelizumab With or Without Probio-M9 in pMMR/MSS Locally Advanced Rectal Cancer

MEDIUM

STAGE IV

GENERAL

NEW TRIAL

CORROBORATED

Mar 9 19:40 · immediate · clinicaltrials.gov

A new clinical trial is exploring the combination of neoadjuvant chemoradiotherapy with the immunotherapy drug Tislelizumab, potentially offering improved treatment options for patients with locally advanced rectal cancer.

1 source(s) reporting · cluster 7 of 8

EVENT CLUSTER — FOLFIRI and Bevacizumab With or Without Pelareorep for Second-Line Treatment of Metastatic RAS-Mutated, Microsatellite-Stable Colorectal Cancer

MEDIUM

STAGE IV

VEGF

NEW TRIAL

CORROBORATED

Mar 9 19:40 · immediate · clinicaltrials.gov

A new trial is exploring whether adding Pelareorep to the standard treatment of FOLFIRI and Bevacizumab can improve outcomes for patients with advanced colorectal cancer that has specific genetic features.

1 source(s) reporting · cluster 8 of 8

Colon Cancer Treatment Reference

Incidence & Trends

~150,000 new US cases/year · ~52,000 deaths/year · 4th most common US cancer

▲ Early-Onset Alert: CRC rates in adults under 50 have risen ~2% per year since the 1990s · Adults under 55 now account for ~20% of all new CRC diagnoses · Cause under active investigation (diet, microbiome, obesity, sedentary lifestyle)

Current Screening Guidelines (2025)

USPSTF / ACS / ACG — Average Risk: Begin screening at age 45 (lowered from 50 in 2021)

Colonoscopy every 10 years · FIT (fecal immunochemical test) annually · Cologuard every 1-3 years · CT colonography every 5 years

High Risk (family history / Lynch syndrome / IBD): Begin at age 40 or 10 years before youngest affected relative — whichever comes first · Colonoscopy every 1-5 years depending on risk

Symptoms at any age: Rectal bleeding, change in bowel habits >4 weeks, unexplained weight loss, iron deficiency anemia → immediate evaluation regardless of age

Key Biomarkers — Test All mCRC at Diagnosis

RAS (KRAS/NRAS) · BRAF V600E · MSI/MMR status · HER2 · NTRK fusion · TMB

Stage IV Targeted Options

KRAS G12C (~3-4%) → sotorasib + cetuximab · adagrasib + cetuximab

BRAF V600E (~8-10%) → encorafenib + cetuximab ± binimetinib

MSI-H/dMMR (~5%) → pembrolizumab (1st line preferred) · nivolumab ± ipilimumab

HER2 amplified (~2-3%) → trastuzumab + pertuzumab · trastuzumab deruxtecan

RAS/BRAF WT → FOLFOX or FOLFIRI + cetuximab or panitumumab (left-sided preferred)

1st Line mCRC Standard of Care

FOLFOX or FOLFIRI ± bevacizumab (RAS mutant / right-sided)

FOLFOX or FOLFIRI + cetuximab or panitumumab (RAS/BRAF WT, left-sided)

Pembrolizumab monotherapy (MSI-H/dMMR)

Key Resources

ClinicalTrials.gov · NCCN Guidelines (nccn.org) · Colorectal Cancer Alliance (ccalliance.org) · Fight Colorectal Cancer (fightcrc.org) · ACS (cancer.org)

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